BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Columns show sequence, not equal lengths. Test timing follows the treatment, clinical findings and local pathway. The treating team owns booking, result review and action when a test is overdue.

Check	Before starting	Chemotherapy visits	Phase change / 12 weeks	Continuation / local care	Completion / follow-up
Blood tests	Baseline FBC, renal and liver profile	FBC, renal and liver checks through chemotherapy; the team confirms the pre-dose schedule	Reassess before a new phase	Companion drugs / abnormal results determine additional tests	Follow persistent abnormalities and the next plan
Echo / cardiac assessment	Baseline echo before anthracycline treatment.	Earlier assessment for symptoms or cardiac concern	Reassess at a clinical / exposure milestone	Repeat according to cardiac risk, symptoms and anthracycline exposure.	ESC supplementary guidance: echo within 12 months after anthracycline completion; high / very-high cardiac risk also needs earlier review, including within 3 months. The team individualises biomarkers and further surveillance.
ECG / rhythm	Baseline ECG in the AC source; repeat for symptoms / cardiac indication. Echo is a separate test.	Symptoms, interacting medicines or clinical indication	An ECG does not replace an echo	Repeat if symptoms or the cardiac plan require it	Agree follow-up if abnormal
Imaging / markers	Document baseline disease and purpose of scans	Review symptoms and response	No routine CT solely because chemotherapy ends	Response imaging guides surgery in neoadjuvant care. Otherwise image for symptoms or a clinical indication.	Annual mammography of remaining breast tissue. No routine CT / PET or tumour markers for asymptomatic distant-recurrence screening.
Bone health / DEXA	Assess endocrine treatment and fracture risk	Not a routine scan at each cycle	Baseline DEXA when indicated	DEXA before aromatase inhibitor / ovarian suppression when indicated; repeat according to fracture risk, results and endocrine plan.	Risk- and result-based follow-up

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00252 v7 · official source · ESMO 2024 early breast cancer guideline · treatment and follow-up · ESC 2022 cardio-oncology guideline · cardiac surveillance and risk · NICE NG101 · bone health and follow-up · ESMO metastatic breast cancer guideline · 2026 · EMA Herceptin product information

THE WHOLE TREATMENT JOURNEY

AC · every 3 weeks: the whole pathway

Adjuvant breast cancer

Doxorubicin and cyclophosphamide on Day 1 every 21 days for four cycles. Any subsequent taxane follows a separately agreed regimen.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before systemic treatment	Chemotherapy phase	Next phase / reassessment	Local / continuing treatment	Follow-up / change
Cancer medicines NCCP 00252	Confirm indication, organ function and access	AC ×4	Review tolerance and response	Complete planned course	Review outcome and next treatment
Surgery EARLY	Early adjuvant: surgery usually already completed	No automatic surgery during these cycles	Reassess local-treatment needs	Pathology / recovery guide next steps	Local follow-up
Radiotherapy EARLY	Assess indication	Usually after chemotherapy in adjuvant care	Coordinate surgery and recovery	Radiation team agrees timing	Assess late effects / local follow-up
Blood tests NCCP 00252	Baseline FBC, renal, liver	FBC, renal and liver checks through chemotherapy; the team confirms the pre-dose schedule	Review before transition	Continue according to active medicines	Follow abnormalities
Heart tests NCCP 00252 · SMPC · ESC	Baseline echo / MUGA	Symptoms can prompt earlier tests	According to cardiac risk	No universal fixed-cycle echo	Agree post-treatment cardiac plan
Scans / markers EARLY · METASTATIC	Baseline staging as indicated	Clinical response / symptoms	Not automatic CT screening	Breast surveillance plan	Symptoms can trigger earlier tests
Endocrine / bone health EARLY · BONE	Check receptors / endocrine indication	Discuss the later plan	Start / continue when agreed	May overlap HER2 / RT if appropriate	Longer-term bone health and endocrine review

What each check is for

1 Blood checks

FBC, renal and liver checks through chemotherapy; the team confirms the pre-dose schedule

Results help the team judge treatment safety.

Ask / act: Confirm the next test and who reviews it.

Sources: NCCP 00252

2 Heart assessment

Baseline cardiac assessment before AC. Repeat imaging according to symptoms, anthracycline exposure and cardiac risk; no universal 12-week echo schedule is assigned to AC alone. ESC guidance supplements the source: baseline echo before anthracyclines; high / very-high cardiac risk may need echo every two cycles and within 3 months after treatment.

Some heart changes occur without symptoms.

Ask / act: Keep the appointment and report new symptoms between scans.

Sources: NCCP 00252 · SMPC · ESC

3 Imaging and markers

Early breast follow-up uses planned breast imaging. Routine CT / PET, blood tumour markers or other distant-recurrence screening are not recommended without symptoms or a clinical indication. After local treatment: annual mammography of remaining breast tissue; additional imaging when indicated.

Each scan should answer a defined clinical question.

Ask / act: Ask when the result will be reviewed.

Sources: EARLY · METASTATIC

4 Booking and handover

At initiation, phase changes and completion

Expected tests can be missed without an owner.

Ask / act: Record booking owner, result reviewer, due window and escalation contact.

Sources: NCCP 00252

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-10-17 · Source reconciled; independent clinical, pharmacy and patient review pending · SACTCheck v0.84.0